



DCC Team TheoVision

GTM Strategy



Introducing Our Team



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The Problem

600M+

people in
developing
countries lack
vision correction

40-

of elderly residents
in facilities have
vision impairment

60%

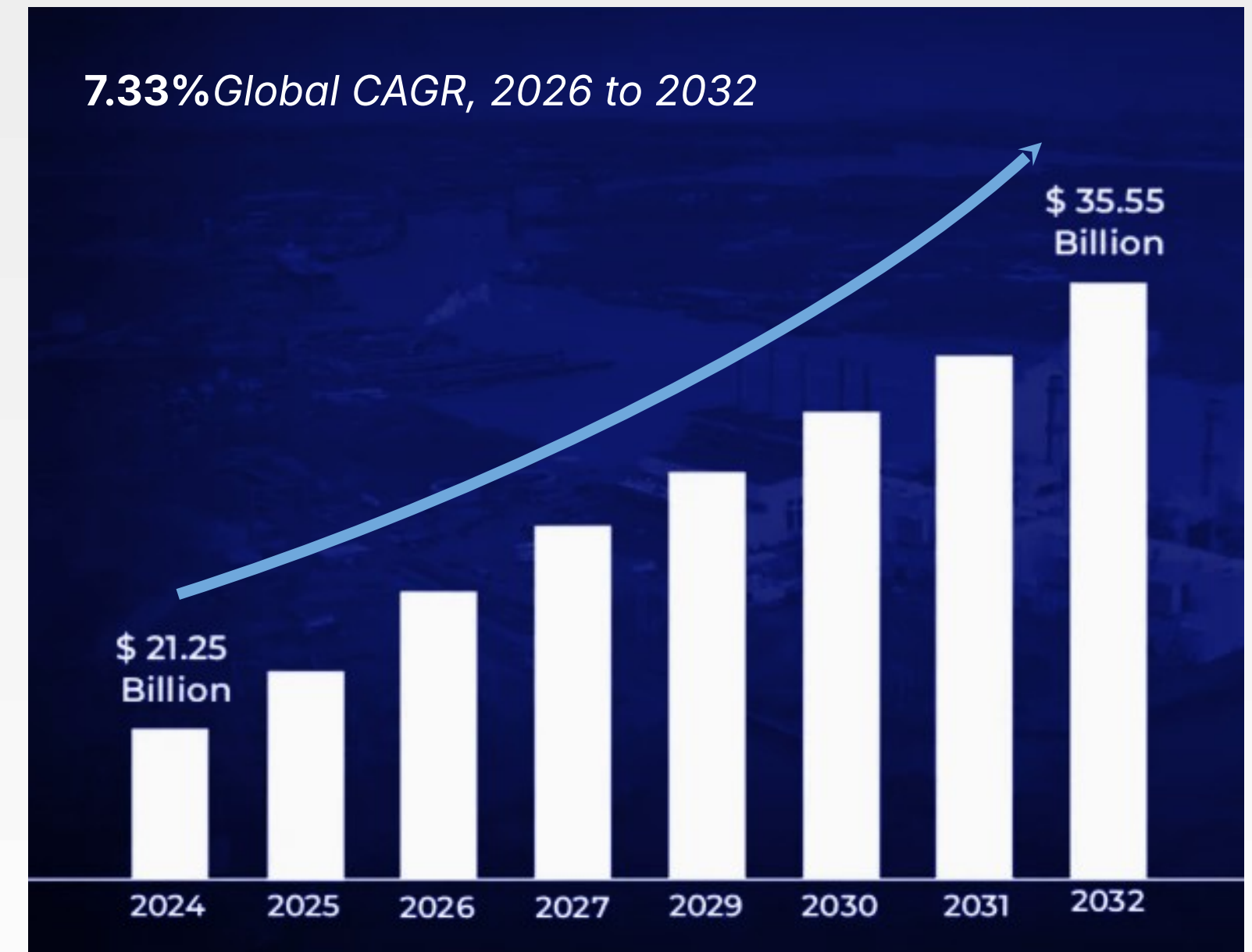
\$200-500

to replace lost or
broken glasses

TheoVision Solution

- Self-correcting glasses kits (~\$100 retail)
- Home-testing → Final glasses at home
- Adjustable lenses for life—kit adjusts to your changing needs
- Affordable, accessible, and immediately available

Optical Lense Market



Per many sources and experts, **global demand for contact, adjustable, and corrective lenses is set to increase in the next decade** due to many external factors such as the recent shift into a more tech & screen based society

Market Potential



Classic Vision Correction

- Highly regulated & takes ample time for FDA approval
- Skepticism of new products by optometrists is high



Sports Eyewear

- FDA approval in an already saturated industry
- Driven largely by partnerships with established athletes



NGO Market

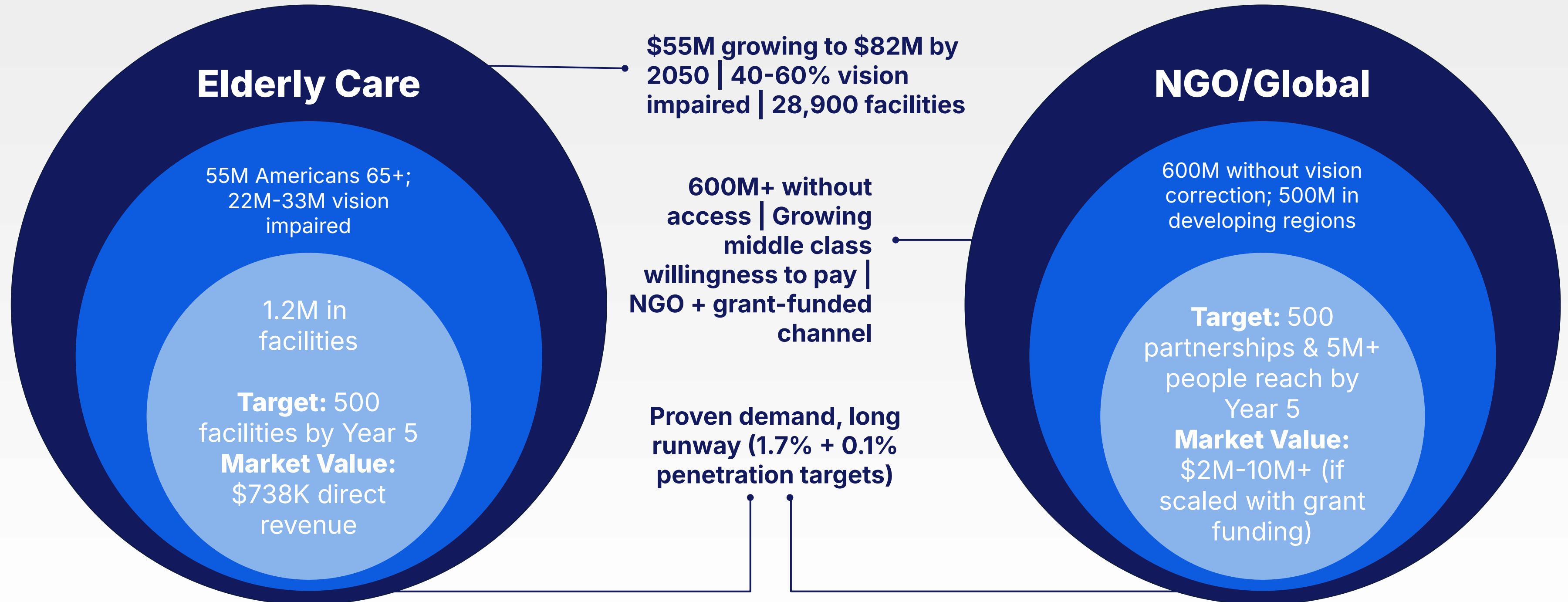
- Minimal FDA regulations with direct global impact
- High potential in low-income areas with mission-driven buyers



Elderly Care

- Rapidly growing elderly demographic with increasing vision needs
- Distribution through retirement homes & care centers

Market Opportunity



Competitive Advantage Chart

	Technology	Price	Prescription Range	Key Limitations	
		\$15-35	Low	+1.25 to +3.00	Readers only
		\$60-120	Low	Limited ranges	Readers only or can't layer
		\$100-150	High	-6D to +3D	Poor aesthetics, limited range
Theovision		\$75-120	Low	0.5D to +12.5D	None identified

Unique value proposition:

- Only stick-on solution for full prescription correction
- Better aesthetics than Adlens (clean cut)
- Versatile application across safety goggles, sports equipment, and sunglasses
- Layerable for extended prescription ranges
- At-home self refraction (no optometrist needed)
- Competitive pricing (\$75-120 vs. \$150-300 for traditional glasses with appointment)

Unit Economics via NGO path

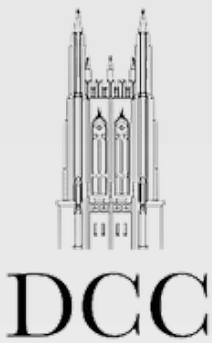


Grant Pays \$0		Grant Pays \$15	
# of Units	100,000	# of Units	100,000
COGS per unit	\$ 15.00	COGS per unit	\$ 15.00
NGO partner payment	\$ 5.00	NGO partner payment	\$ 5.00
Proposed Grant subsidy	\$ 0.00	Proposed Grant subsidy	\$ 15.00
Revenue per unit	\$ 5.00	Revenue per unit	\$ 20.00
Variable cost/unit	\$ 0.00	Variable cost/unit	\$ 0.00
Total Variable cost/unit	\$ 15.00	Total Variable cost/unit	\$ 15.00
Unit contribution margin	\$ (10.00)	Unit contribution margin	\$ 5.00
Total Revenue	\$ 500,000.00	Total Revenue	\$ 2,000,000.00
Total COGS	\$ 1,500,000.00	Total COGS	\$ 1,500,000.00
Total Variable Costs	\$ 1,500,000.00	Total Variable Costs	\$ 1,500,000.00
EBITDA	\$ (1,000,000.00)	EBITDA	\$ 500,000.00

- Considering a \$15 COGS, an NGO model is **only profitable with grant funding of \$10 or more per unit**
- Scale potential: 100K-500K+ units annually
- Capital efficient: Grants fund this channel, no dilution to your equity
- Important to consider COGS as a potential variable factor

Subsidy Value Per Unit	\$ 0.00	\$ 5.00	\$ 10.00	\$ 15.00	\$ 20.00
Unit Contribution Margin	\$ (10.00)	\$ (5.00)	\$ 0.00	\$ 5.00	\$ 10.00
Annual Profit	\$ (1,000,000.00)	\$ (500,000.00)	\$ 0.00	\$ 500,000.00	\$ 1,000,000.00

Unit Economics via Elderly Care Market



Per-Kit Economics

Metric	Value
Wholesale price	\$70
COGS	\$28 (→ \$22 by Yr 3)
Gross profit	\$42 (60% margin)

Order-Level/Annual

Metric	Value
Gross profit per 50-kit facility order	\$2,100
Annual per facility	\$2,100 - \$4,200

Refills

Metric	Value
Refill price	\$20
Refill COGS	\$5
Refill gross profit	\$15 (75% margin)

Takeaways:

- Facilities willing to pay \$70/kit (vs. \$200-500 traditional replacement cost)
- 60% gross margin sustainable TODAY (don't need scale to work)
- Recurring revenue: Refills at \$20 with 75% margin
- LTV per facility: \$17,700 over 5 years | CAC: \$1,500 | Payback: 4-5 months

Why NGOs?

- **Lower barrier for entry**

- Easier for TheoVision to enter market without regulatory approval

- **Existing distribution networks**

- NGOs already operate in underserved communities
- No need for TheoVision to build local infrastructure

- **Mission alignment → easier funding**

- Mission to increase eye health access = ideal grant recipient
- Will reduce COGS + finance large scale deployments

- **Credibility & trust-building**

- Partnerships strengthen positioning with global institutions

- **Lower CAC & efficient scale**

- Eliminate need for consumer acquisition
- Bulk distribution unlocks immediate reach to 100k - 500k+ people annually

Target NGOs:



- Operates in 30+ countries
- Large track record for refractive error programs



- High-volume distributor to low-income populations
- Ideal for bulk procurement pilots



- Strong surgical and refractive programs
- Ideal for grant-supported expansion

Edu. NGOs



- Core partners for UNICEF Venture Fund school deployment model
- Enable national educational ministry adoption

Why Elderly Care?

● Strong market demand



- Elderly care facility residents face 40-60% vision impairment
- Pressing need for low cost, rapid solutions

● Attractive unit economics



- Facilities willingly pay \$70 per kit as alternatives are steeper
- ~60% gross margins without requiring large scale sales

● Recurring revenue potential



- \$20 refills with 75% margin → predictable recurring income
- 4-5 month CAC payback

● Clear path to scalable adoption



- Highly centralized market of ~29,800 facilities
- Efficient outreach through existing networks
- Viable growth to 500+ facilities within 5 years

1. Why Start With NGOs/Grants?

- No need for regulatory approval
- Does not require optometrist involvement
- Cheaper, less risky, and quicker way to get the product to market
- Opportunity to validate product and generate impact data in low stakes environment
- Builds credibility + manufacturing readiness

2. Why Elderly Care Second?

- Generates more stable revenue
- Large market with high demand and growth opportunities
- Higher margins, recurring revenue, and scalable adoption

NGO Partnership Models



Bulk Procurement and Distribution

- NGO purchases our product in bulk at tiered discounted prices
- Distributes our kits through existing local networks

Joint Program Delivery

- Vision accessibility program co-designed with NGO
- TheoVision supplies kits
- NGO handles community access and education

Telehealth-Enhanced Model

- TheoVision provides vision correction kits
- NGO/telehealth partner offers disease screening, support for client concerns, and referrals to local clinics

What We Envision:

- A community-driven program that does not require access to an optometrist
- A hybrid model combining our physical kits with telehealth support for medical issues
- A low cost solution for communities in need that NGOs can sustain through grants
- Expansion across regions and countries due to reliability and partnerships

5 year projection (NGO)

Year 1

- Initial pilot with NGO
- NIH/NEI grant lowers COGS to ~\$6-8
- Validate accuracy + generate baseline data

Year 2

- Expand to 2-3 NGO partners
- NSF SIBR redesign → COGS ~\$3-5
- Add Red-flag telehealth screening

Year 3

- UNICEF-supported adoption via Ministries
- Shift to bulk procurement; multinational rollout

Year 4

- Secure \$200k-500k grants
- Local NGOs operate, TheoVision supplies + trains
- \$60-80 NGO price tiering becomes sustainable

Year 5

- VCF partnership enables large scale deployment
- National school programs
- TheoVision is preferred refractive supplier for LMIC NGOs

5 year projection (Elderly)



Year 1

- Launch pilots; validate cost savings and ease of use
- ~\$3.5k revenue per facility; 60% gross margin

Year 2

- Expand through LeadingAge/Argentum channels
- Begin recurring refills (75% margin); LTV/CAC viable

Year 3

- COGS drop from \$15 → ~\$10; margins strengthen
- Refills becoming meaningful recurring income

Year 4

- Win regional chains + multi-site operators
- Maintain low CAC while scaling profitable accounts

Year 5

- Reach full target: 500 facilities served
- Strong recurring income + ~\$8.5+ portfolio LTV

Recommended Approach

1

NGO + grant model

Leverage NGO networks and grant subsidies together to deliver affordable, scalable, and clinically validated vision correction.

2

Telehealth partnership

Integrate a remote screening partner to expand clinical capability, support red-flag screening, and strengthen credibility.

3

Distribution channels + operations refinement

Expand distributor network while refining training and workflow, and minimize COGS through improved sourcing, manufacturing, and scale optimization.

4

Elderly care model

Prioritize elderly care as a high-margin, fast payback channel, scaling to 500+ facilities with strong recurring revenue.